



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : انتصار عبدالحميد محمد عبدالهادي

المحافظة : القاهرة الحي / المدينة : مصر القديمه العنوان : 0

رقم البطاقة : 27402021402004 رقم التليفون : 01270702126 تاريخ تسجيل الإستمارة : 21-09-2026

العمر (تاريخ الميلاد) : 02-02-1974 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /د. /د. /د. /د. /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: انتصار عبد الحميد محمد عبد الهادي National ID: 27402021402004	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



8/16/2026

MDT FORM

Name: Entisar Abdelhamid Mohamed Abdelhady	MRN: 6509
Age: 53 Y	ID: 27402021402004
Menopausal Sta	Phone: 01270702126
NRS SCORE:	Marital Status:
BMI:	Family History:
CO Morbidity:	Reason Of MDT:

Clinical Data:

BX: **R** **L**

ER: **PR:** **HER2:** **KI67:**

Sonomammography:**Mets Workup:****History:**

prem rt breast cancer multicentric ct2 no mx initial cts pleural based pulmonary nodules largest 1.2 cm (suspicious) rec taxol and herceptin rec 18 weeks taxol 7 cycles herceptin .cts 1/2026 free no pulmonary nodules pt underwent rt SM + SLNB (POST NACT) 6/5/26 minimal residual IDC G2 free margins SLNB 0/6 one LN show therapy effect ypT 1a ypN 0 (sin) patient didnt receive herceptin since 4/26 new ctscan showed subpleural enhancing (reappeared)

newly developed foci of osteolysis scattered in th spin

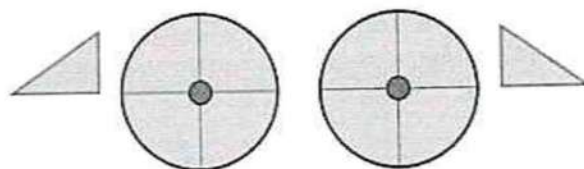
for bone scan

Recommendations:

for xeloda/tykerb

MDT Outcome & Plan

for xeloda/tykerb

**T N M****SIGNATURE**

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. حسان عزت					

Patient's Name: Intisar Abdulhamid Mohammed Abdelhadi
Date: 2026-08-02
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



MULTISLICE CT EXAMINATIUNON OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- Evident mastectomy of the right breast with no evidences of either residual/recurrent neoplastic lesions seen herein.
- **Normal left breast.**
- **Benign looking left axillary lymph nodes** are seen with preserved their hilar fat contents and the largest one measures about 10 mm.
- Hyperinflated both lungs.
- Subpleural enhancing pulmonary nodules are reappearing(1 in the anterior segment of right upper pulmonary lobe and measures about 12mm and three in the corresponding left upper lobe,with calcification noted in one of them and the largest one measures about 13mm.
- Thin linear atelectatic bands are seen in both lower pulmonary lobe.
- No other sizable abnormally-enhancing pulmonary nodules ,patchy areas of consolidations or foci of calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusions.
- Average size of the heart with normal CT of its big vessels.
- Newly-developed ill-defined foci of osteolysis are seen scattering in the scanned spine.
- Increased size of left lobe and average sized right lobe of the beaver-tail liver, showing a normal parenchymal attenuation and enhancement pattern.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 11.3 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal glands(**apart from a left parapelvic region and measures about 39x28mm in dimensions**), aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- An IUCD is seen in the uterine cavity.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.

IMPRESSIONS:

A status follow up ,compred with the previous study done at 12/01/2026, the current examination shows:

Patient's Name: Intisar Abdulhamid Mohammed Abdelhadi
Date: 2026-08-02
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- Evident right mastectomy with no evidences of either residual/recurrent neoplastic lesions seen herein.
- Hyperinflated both lungs, with subpleural enhancing pulmonary nodules reappearing(1 in the anterior segment of right upper pulmonary lobe and three in the corresponding left upper lobe, with calcification noted in one of them) and newly-developed ill-defined foci of osteolysis scattering in the scanned spine, denoting metastases.
- Thin linear atelectatic bands in both lower pulmonary lobe.

Prof/Hanan El-Ahwal.

Patient Name : الأستاذة / انتصار عبد الحميد محمد عبد الهادي
Patient No. : 27402021402004
Path No. : 1377/26
Sex : Female
Age : 52 Y
Referreing Doc :

Request No. : 13006369
Request Date : 07/05/2026
Verification Date : 24/05/2026
Financial Category : جوستاف روسي - مبادره
Department : Out Patient
Registered by : Ali mostafa

Pathology Report

Clinical Data: Right breast cancer (Post-NACT).

Nature of specimen: Right simple mastectomy & Sentinel lymph nodes biopsy.

Gross Examination:

- * Right simple mastectomy specimen measured 22x10x5 cm, with covering skin ellipse 17x10 cm. Nipple & areola looked unremarkable grossly.
- * Serialling showed a tumor bed site measured 9x4x3 cm, located 0.5 cm from the least deep margin.
- * Sentinel lymph nodes: fibrofatty tissue piece measured 7x6 cm, entangling 6 lymph nodes, the largest measured 2x1 cm.
- * 12 Blocks:
1:9- TBS., 10- Nipple., 11- Deep margin ., 12- SLNs.
- Gross was done by; Dr. Heba El Shafeey.
- ** Frozen (Intra-operative) results:
- SLNs: Six free sentinel lymph nodes (0/6).
- Frozen examined by Prof. Dr. Maha Guimei.

Microscopic Examination:

- * Histological Type: Minimal residual invasive duct carcinoma
- * Size of largest invasive focus: 0.5x0.5 cm Cellularity (4%) .
- * Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- * Angioinvasion: Absent.
- * Ductal carcinoma in situ: 5% Intermediate grade DCIS (Solid, cribriform & clinging patterns).
- * Microcalcification: Present.
- * Nipple & areola: Free.
- * Deep margin: Free.
- * Sentinel lymph nodes: Six lymph nodes free of metastatic deposits (0/6). One lymph node

Verified by :

Dr Maha Guimie



Patient Name	: الأستاذ / انتصار عبدالحميد محمد عبدالهادي	Request No.	: 13006369
Patient No.	: 27402021402004	Request Date	: 07/05/2026
Path No.	: 1377/26	Verification Date	: 24/05/2026
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 52 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Ali mostafa

Pathology Report

showing therapy effect.

* Histologic response to therapy: Pathological partial response to therapy.

Diagnosis:

Right breast cancer (Post-NACT), Right simple mastectomy & Sentinel lymph nodes biopsy:

- Minimal residual viable invasive duct carcinoma, Grade 2.
- Sentinel lymph nodes: Six lymph nodes free of metastatic deposits (0/6) with therapy effect.
- Deep margin: Free.
- Pathological partial response to therapy.
- Stage: ypT1a, ypN0 (sln).
- RCB: 1.056, RCB class I.

Signature

Dr.Walaa Ibrahim Ghoneim

Verified by :

Dr Maha Guimie

Printing Date : 24/05/2026 10:08:16

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Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / انتصار عبد الحميد محمد عبدالهادي
Patient No. : 27402021402004
Path No. : 928/25
Sex : Female
Age : 51 Y
Referring Doc :

Request No. : 13007530
Request Date : 05/03/2025
Verification Date : 22/03/2025
Financial Category : جوستاف روسي - مياثره
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data
Right breast mass

Nature of specimen
Tru cut biopsy, collected at 5/3/2025, 10:30 am

Gross examination

Multiple grayish white tissue cores up to 1 cm, totally submitted.
IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade II
- * DCIS: Positive
- * Angioinvasion: Couldn't be detected in the received biopsy
- * Perineural invasion: No
- * IPT (Positive control showed appropriate reactivity) :
- * E-cadherin: Positive
- * ER: Moderate nuclear reaction in 70 %tumor cells (Allred score 7/8)
- * PR: Moderate nuclear reaction in 40 %tumor cells (Allred score 6/8)
- * HER2: Strong complete membrane reaction in >10 % of tumor cells (score 3+)
- * Ki 67: 11 % nuclear reaction of tumor cells

Diagnosis

Verified by :
Dr.Heba Hasan

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Sex : Female
Age : 51 Y
Referreing Doc :

Request No. : 13007530
Request Date : 05/03/2025
Verification Date : 22/03/2025
Financial Category : جوسفاف روعي - مبادره
Department : Out Patient
Registered by : .marwa mohamed youss

Pathology Report

Right breast mass, Tru cut biopsy:
Invasive duct carcinoma, Grade II
IPT:

- * ER: Positive (score 7/8).
- * PR: Positive (score 6/8).
- * HER2: Positive (score 3+).
- * Ki67: Positive in 11% of tumor cells.

Signature
Prof. Dr. Lubna Omar
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name : ENTESAR ABDELHAMEED

Date : 12-01-2026 12:27

MOHAMED

Branch : Portable

Medical No. : GRI-98110

Ref.Physician Prof :

Ins.Company :

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

I- CHEST:

- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulæ or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

❖ No hepatic or pulmonary metastatic lesions.

Dr. Asser Ahmed

Dr. Asser Ahmed